



PEPTIDE**PROTOCOL**PORTAL

CLINICAL INTELLIGENCE • PRECISION PROTOCOLS

MODULE 15 • MODULAR FORM SET

FDA-Approved GLP-1

Receptor Agonist Consent

Companion consent for branded and generic semaglutide, tirzepatide, liraglutide, dulaglutide & exenatide

MODULAR FORM SET

PEPTIDEPROTOCOLPORTAL.COM

Practice & Clinical Information

Clinic / Practice Name

Practice Address

State of Care

Supervising / Prescribing Physician

State License #

Telephone (Business Hours)

After-Hours / Urgent Line

Patient Portal / EHR

Pharmacy / Supplier of Record

Patient Identification

Patient Legal Name (First, Middle, Last)

Date of Birth (MM/DD/YYYY)

Sex Assigned at Birth

Patient ID / MRN

Mailing Address

Mobile Phone

Email

Emergency Contact Name

Relationship & Phone

Primary Care Physician (Name & Phone)

PLEASE READ CAREFULLY BEFORE SIGNING

This document is a comprehensive informed-consent agreement and patient educational manual. It explains, in plain language, what peptide therapy is, the regulatory and clinical context of these therapies, the risks of self-administration in your home, and your responsibilities as a patient electing to participate. You are encouraged to read every section, ask any questions, and decline to proceed at any time without penalty.

Sections marked with an initials line require your individual initials to confirm you have read, understood, and agree to that specific item. Signatures appear on the final pages.

1. Purpose, Scope & Voluntary Election — FDA-Approved GLP-1 Receptor Agonist Therapy

What This Consent Covers — and What It Does Not

This consent applies specifically to FDA-approved branded or generic GLP-1 receptor agonist medications dispensed through a licensed pharmacy as approved drugs, including:

- Semaglutide — Ozempic®, Wegovy®, Rybelsus® (oral)
- Tirzepatide (a dual GIP/GLP-1 receptor agonist) — Mounjaro®, Zepbound®
- Liraglutide — Victoza®, Saxenda®
- Dulaglutide — Trulicity®
- Exenatide — Byetta®, Bydureon® BCise®

This consent does NOT cover compounded GLP-1 / GIP-GLP-1 formulations (e.g., compounded semaglutide or compounded tirzepatide). Compounded versions of these molecules are governed by the master injectable peptide consent and must be addressed separately by my practice. The regulatory status, supply chain, and risk discussion for compounded versions differ from the branded approved drugs.

I, the undersigned patient, am voluntarily electing to receive an FDA-approved GLP-1 receptor agonist medication as part of an individualized clinical program designed by my treating provider. The medication will be dispensed through a licensed pharmacy in accordance with my prescription.

I understand the medication may be prescribed for an FDA-approved indication (e.g., type 2 diabetes mellitus, chronic weight management in eligible patients, cardiovascular risk reduction in select products) or for an off-label indication recognized by the practice of medicine. My provider has discussed which indication applies to me.

2. Regulatory Status, FDA-Approved Indications & Off-Label Use

I understand and acknowledge:

Initials _____ The medication I am receiving is an FDA-approved drug product for one or more specific indications. I have been informed of the indication for which my prescription is being written.

Initials _____ If my prescription is being written for an off-label indication (an indication other than the one approved by the FDA for this drug), I understand that off-label prescribing is a lawful and recognized component of the practice of medicine but does not carry FDA endorsement for that specific use.

Initials _____ FDA-approved drugs carry product labeling that includes Boxed Warnings, contraindications, warnings and precautions, adverse reactions, and dosing information. I have received or have been offered the official Medication Guide for my specific product, and I have been encouraged to read it.

Initials _____ This medication is being dispensed through a licensed pharmacy. Refills, dose escalations, and interruptions in supply may occur due to manufacturer back-orders or insurance authorization decisions; I will work with my provider and pharmacy on continuity of therapy.

Initials _____ This consent is companion to — not a substitute for — the FDA Medication Guide and the prescribing information distributed with the product.

3. Boxed Warnings & Major Safety Disclosures

BOXED WARNING — Risk of Thyroid C-Cell Tumors

GLP-1 receptor agonists (including semaglutide and tirzepatide) carry a Boxed Warning for the risk of thyroid C-cell tumors based on rodent studies. The relevance of these findings to humans has not been determined. These medications are CONTRAINDICATED in patients with:

- A personal or family history of medullary thyroid carcinoma (MTC)
- Multiple Endocrine Neoplasia syndrome type 2 (MEN-2)

I will inform my provider immediately of any neck mass, hoarseness, dysphagia, or persistent shortness of breath.

Initials _____ I confirm I have no personal or family history of MTC or MEN-2.

3.1 Pancreatitis

Acute pancreatitis, including fatal cases, has been reported in patients on GLP-1 receptor agonists. I will discontinue the medication and seek immediate medical attention for severe, persistent abdominal pain (with or without vomiting), particularly pain that radiates to the back.

Initials _____ I understand the pancreatitis warning and the symptoms that require immediate evaluation.

3.2 Acute Kidney Injury

Acute kidney injury, sometimes requiring dialysis, has occurred — usually in the setting of nausea, vomiting, diarrhea, and dehydration. I will maintain adequate hydration and report persistent GI symptoms or decreased urine output.

Initials _____ I understand the kidney injury warning and the importance of hydration.

3.3 Diabetic Retinopathy Complications (Semaglutide)

Patients with type 2 diabetes and a history of diabetic retinopathy may experience worsening of retinopathy, particularly with rapid improvement in glucose control. I will continue routine eye examinations as advised.

Initials _____ I understand the retinopathy warning and will continue eye-care follow-up as directed.

3.4 Hypoglycemia

GLP-1 receptor agonists by themselves carry low intrinsic hypoglycemia risk. The risk increases significantly when used with insulin, sulfonylureas, or other glucose-lowering agents. My provider may adjust those concurrent medications.

Initials _____ I understand the hypoglycemia risk and will monitor glucose if I am also taking insulin or a sulfonylurea.

3.5 Severe Gastrointestinal Disease

Severe GI adverse reactions have been reported, including ileus and gastroparesis. Patients with pre-existing severe gastroparesis or significant GI motility disorders are not appropriate candidates.

Initials _____ I understand the severe GI disease warning.

3.6 Gallbladder Disease

Cholelithiasis and cholecystitis have been reported, particularly during rapid weight loss. I will report right-upper-quadrant pain, fever, or jaundice promptly.

3.7 Suicidal Behavior and Ideation

Post-marketing reports have prompted regulatory review of possible suicidal ideation in patients on GLP-1 receptor agonists. I will report new or worsening depression, mood changes, or thoughts of self-harm to my provider immediately and use 988 or the nearest emergency department for active suicidal thoughts.

Initials _____ I understand the suicidal-ideation safety signal and the escalation pathway.

3.8 Pregnancy & Reproductive Considerations

GLP-1 receptor agonists are not recommended during pregnancy. A washout period before attempting conception is typically advised — semaglutide is generally discontinued at least 2 months before a planned pregnancy due to its long half-life. Patients of reproductive potential should discuss reliable contraception with their provider before initiating therapy. Patients on combined oral contraceptives should be aware that significant GI effects may reduce contraceptive efficacy; barrier or non-oral contraception may be advised.

Initials _____ I understand the pregnancy considerations, washout requirement, and contraceptive guidance.

4. Surgery, Anesthesia & Procedural Considerations

Pre-Procedure Notification — Important

Because GLP-1 receptor agonists slow gastric emptying, the American Society of Anesthesiologists (ASA) has issued guidance recommending that GLP-1 RAs be held prior to elective procedures requiring sedation or general anesthesia, due to the risk of retained gastric contents and aspiration during anesthesia.

If you are scheduled for any of the following, notify both the procedural team and my practice well in advance so a hold plan can be coordinated:

- Elective surgery (any type)
- Endoscopy, colonoscopy, or any procedure requiring sedation
- Imaging requiring sedation (e.g., MRI in select patients)
- Dental procedures requiring sedation
- Labor and delivery — discuss anesthesia plan during prenatal care if applicable

Initials _____ I will inform every clinician who treats me — including surgeons, anesthesiologists, dentists, gastroenterologists, urgent-care providers, and emergency-department staff — that I am taking an FDA-approved GLP-1 receptor agonist.

Initials _____ I will notify the practice in advance of any planned procedure so a hold strategy can be developed.

5. Patient Health Disclosure & Eligibility Screening

I have truthfully disclosed each of the following to my provider, and I will report any change:

Disclosure Item	Yes	No	Notes
Personal or family history of medullary thyroid carcinoma (MTC)	☐	☐	
Multiple Endocrine Neoplasia syndrome type 2 (MEN-2)	☐	☐	
Personal history of pancreatitis	☐	☐	
Gallbladder disease, biliary disorders, or recent gallbladder surgery	☐	☐	
Severe gastroparesis or GI motility disorder	☐	☐	
Active or recent eating disorder (anorexia, bulimia, ARFID, BED)	☐	☐	
Type 1 diabetes or history of diabetic ketoacidosis	☐	☐	
Diabetic retinopathy (any stage)	☐	☐	
Chronic kidney disease or impaired renal function	☐	☐	
Hepatic disease or impaired liver function	☐	☐	
Use of insulin or sulfonylureas	☐	☐	
Concurrent use of any other GLP-1 / GIP receptor agonist (branded or compounded)	☐	☐	
History of depression, suicidal ideation, or other significant psychiatric condition	☐	☐	
Pregnant, planning pregnancy within 2 months, or breastfeeding	☐	☐	
Use of oral contraceptives (efficacy may be reduced by significant GI effects)	☐	☐	
Planned surgery or procedure with sedation in the next 90 days	☐	☐	
Significant or uncontrolled cardiovascular	☐	☐	

Disclosure Item	Yes	No	Notes
disease			

Eligibility Notes (Provider-Completed)

Indication for therapy (FDA-approved or off-label)

BMI / weight criteria documented? (Y/N + value)

A1c / glucose criteria documented? (Y/N)

Baseline labs reviewed (CMP, lipid panel, A1c, lipase if indicated)

Baseline weight / waist / blood pressure / heart rate

Initials _____ I have completed the disclosures above truthfully and will notify the practice of any changes.

6. Administration, Dose Escalation, Storage & Pen Handling

6.1 Pre-Filled Pen Devices

FDA-approved injectable GLP-1 receptor agonists are typically supplied in pre-filled pen-injector devices. These devices have specific instructions for activation, needle attachment, dose-dial selection, injection technique, and disposal. The Manufacturer Instructions for Use included with the device are the authoritative reference for handling, and patients should review them at every refill in case of changes.

- Use a NEW pen-needle for each injection; never re-use a needle.
- Inspect the medication for clarity and absence of particles before each use; discard if cloudy or discolored.
- Rotate injection sites — typical sites are abdomen (avoid 2 inches around navel), upper outer thigh, or upper outer arm.
- Hold the pen in place for the full count specified by the manufacturer (commonly 6–10 seconds) after pressing the dose button to ensure complete delivery.
- Dispose of pen-needles immediately into an FDA-cleared sharps container; do not place sharps in household trash or recycling.
- Discard the pen device per the manufacturer's beyond-use guidance (e.g., many products are discarded 28–56 days after first use, even if medication remains).

6.2 Oral Semaglutide (Rybelsus®) — Special Administration

- Take in the morning on an empty stomach with no more than 4 ounces of plain water — at least 30 minutes before the first food, beverage, or other oral medication of the day.
- Swallow the tablet whole — do not split, crush, or chew.
- Other oral medications taken too close to the dose may have altered absorption.

6.3 Dose Escalation

FDA-approved GLP-1 receptor agonists are typically initiated at a low dose and titrated upward over weeks to months to reduce GI side effects. I will follow the escalation schedule provided by my provider and will not advance my dose ahead of schedule.

Initials _____ I will follow the dose-escalation schedule exactly as written by my provider.

Initials _____ I will not double a missed dose; I will follow the manufacturer's missed-dose guidance for my specific product or contact the practice.

6.4 Storage

State	Storage	Notes
Unopened pen / sealed product	Refrigerated 2–8 °C; protect from light	Do not freeze; do not use if frozen
In-use pen	Per manufacturer label — many can be at room temperature once started	Discard at the manufacturer's BUD (often 28–56 days)
Travel	Insulated cooler with ice pack; avoid direct contact with ice	Same-day transit; refrigerate on arrival
Oral tablets (Rybelsus®)	Original blister pack at room temperature	Per labeling

7. Common Adverse Effects, Tolerability & What to Expect

7.1 Most Common Adverse Effects (Especially During Initiation and Dose Escalation)

- Nausea — particularly with the first dose at each new strength
- Vomiting
- Diarrhea or constipation
- Abdominal pain or dyspepsia
- Decreased appetite (a therapeutic effect, but can be excessive)
- Injection-site reactions — pain, redness, itching, or small nodule
- Headache
- Fatigue
- Dizziness

7.2 Tolerability Strategies

- Eat smaller, more frequent meals.
- Avoid large, high-fat, or greasy meals.
- Stop eating when full — do not push past satiety.
- Maintain hydration with water and electrolytes.
- Maintain adequate protein intake to preserve lean mass.
- Consider a brief dose-hold or extended titration interval if GI effects are intolerable; coordinate with your provider before adjusting.

7.3 Less Common but Important Effects (Contact Provider)

- Persistent vomiting, dehydration, or inability to keep fluids down
- Significant weight loss exceeding the planned trajectory
- Loss of lean body mass or muscle weakness
- Hair shedding (often related to rapid weight loss; usually self-limited)
- Mood changes, sleep changes, or new psychiatric symptoms
- Persistent constipation requiring intervention
- Gallbladder symptoms (RUQ pain, fever, jaundice)
- Eye changes, especially in patients with diabetic retinopathy
- Cardiovascular symptoms — palpitations, chest discomfort

EMERGENCY WARNING — Call 911 or Go to the Nearest Emergency Department

- Severe, persistent abdominal pain, with or without vomiting (possible pancreatitis)
- Difficulty breathing, throat tightness, or facial / lip / tongue swelling
- Severe dehydration, decreased urine output, or signs of acute kidney injury
- Active suicidal thoughts or new severe mood disturbance
- Signs of severe allergic reaction or anaphylaxis
- Severe sudden vision change
- Signs of bowel obstruction — severe abdominal distension, intractable vomiting

8. Voluntary Election, Acknowledgments & Signatures

Please initial each statement to confirm your understanding:

Initials _____ I have read this FDA-Approved GLP-1 Receptor Agonist Consent in its entirety or had it read to me, and the language used is one I understand.

Initials _____ I have had the opportunity to ask questions, and my questions have been answered to my satisfaction.

Initials _____ I understand my prescription is for an FDA-approved branded or generic GLP-1 receptor agonist, dispensed through a licensed pharmacy.

Initials _____ I understand and accept the Boxed Warnings and major safety disclosures, including thyroid C-cell tumor risk, pancreatitis, acute kidney injury, retinopathy, hypoglycemia (when combined with insulin or sulfonylureas), severe GI events, gallbladder disease, suicidal behavior signal, and pregnancy considerations.

Initials _____ I understand the surgical / anesthesia consideration and will notify all procedural teams in advance.

Initials _____ I will follow only the dose-escalation, administration, and missed-dose instructions provided by my treating provider and the FDA Medication Guide.

Initials _____ I will report adverse effects promptly and seek emergency care for red-flag symptoms.

Initials _____ I am 18 years of age or older (or the age of consent for this product per labeling) and competent to give informed consent.

Initials _____ My election to use this medication is voluntary, and I assume the risks set out in this document and the FDA-approved labeling.

Patient — by signing below, I confirm I have read, understood, and voluntarily agreed to all terms of this FDA-Approved GLP-1 Receptor Agonist Therapy Consent.

Patient Signature

Date

Patient Printed Name

DOB

Provider — by signing below, I confirm that the patient was screened for contraindications and Boxed Warning conditions, was determined an appropriate candidate per FDA-approved or recognized off-label indication, was provided the FDA Medication Guide for the specific product, was instructed in pen-device or oral administration as appropriate, and had questions answered.

Provider Signature

Date

Provider Printed Name & Credentials

License #

Specific Product Prescribed (drug, brand, strength)

Prescribed Dose & Titration Schedule

FDA Medication Guide Provided to Patient (Date)

Pharmacy Dispensing



PEPTIDE PROTOCOL PORTAL

Clinical Intelligence • Precision Protocols

ABOUT THIS DOCUMENT

This documentation package is part of the Peptide Protocol Portal — a clinician-facing resource for evidence-informed peptide therapy. It is intended to be reviewed by qualified legal counsel and the supervising physician of the deploying practice prior to clinical use.

DOCUMENT VERSION v3.0

Reviewed prior to deployment by Medical Director and Healthcare Counsel

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